

Important information. Please read carefully.

eze[®]

Tablet

COMPOSITION

Each tablet contains Loratadine 10 mg.

MODE OF ACTION

Loratadine is a long-acting tricyclic antihistamine with selective peripheral histamine H₁-receptor antagonistic activity. It has little central sedative or antimuscarinic activity.

PHARMACOKINETICS

Loratadine is rapidly absorbed from the gastro-intestinal tract after oral administration, peak plasma concentrations being attained in about one hour. Systemic bioavailability (AUC) is increased and time to peak plasma concentration (T_{max}) is delayed when administered with food. Peak plasma concentrations (C_{max}) were not affected by food. Loratadine undergoes extensive metabolism. The major metabolite, descarboethoxyloratadine, has potent histamine-H₁ blocking activity. Reported half-lives for loratadine and descarboethoxyloratadine are 12 and 18 hours respectively. Loratadine is about 98% bound to plasma proteins; descarboethoxyloratadine is less extensively bound. Loratadine and its metabolites have been detected in breast milk, and are excreted in the urine and faeces.

INDICATION

eze[®] Tablet is indicated for the relief of symptoms associated with allergic rhinitis, including sneezing, nasal discharge and itching, as well as ocular itching. It is also indicated for the relief of symptoms associated with chronic urticaria and other allergic dermatologic disorders.

DOSAGE AND ADMINISTRATION

Adults and children 12 years old and over:

One **eze[®]** Tablet (10 mg) once daily.

Children 6 to 12 years old:

Body weight > 30 kg - One tablet (10 mg) once daily.

Body weight < 30 kg - Half a tablet (5 mg) once daily.

CONTRAINDICATIONS

eze[®] Tablet is contraindicated in patients who are hypersensitive to this medication or any of its ingredients.

WARNINGS AND PRECAUTIONS

Patients with liver impairment or renal insufficiency (GFR < 30 mL/min) should be given a lower initial dose because they have reduced clearance of loratadine. An initial dose of 5 mg once daily or 10 mg every other day is recommended.

Use in Pregnancy and Lactation

Warning: Safe use of this product during pregnancy has not been established. There are no adequate and well-controlled studies in pregnant women; therefore **eze[®]** Tablet should be used during pregnancy only if clearly needed. Since loratadine and descarboethoxyloratadine pass easily to breast milk and because of the increased risk of antihistamines for infants, particularly newborns and premature infants, a decision has to be made whether to discontinue nursing or to discontinue the drug, taking into account the importance of the drug to the mother.

Use in Paediatric

The safety and effectiveness of **eze[®]** Tablet in paediatric patients under 6 years of age have not been established.

DRUG INTERACTIONS

Increased plasma concentrations of loratadine and/or descarboethoxyloratadine were reported when administered concomitantly with erythromycin, cimetidine and ketoconazole, but without clinically significant changes in the safety profile of loratadine, as assessed by electrocardiographic parameters, clinical laboratory tests, vital signs and adverse events. When administered concomitantly with alcohol, loratadine has no potentiating effects as measured by psychomotor performance studies. Co-administration with drugs that inhibit or are metabolised by hepatic cytochrome P450 3A4 or 2D6 may elevate plasma concentrations of either drug and adverse reaction might result.

SIDE EFFECTS / ADVERSE REACTIONS

Most common side effects associated with loratadine include headache, fatigue and somnolence, dry mouth, gastrointestinal disorders such as nausea, gastritis, and allergic symptoms like rash. There are no problems with sedation or antimuscarinic activity in usual doses. Alopecia, anaphylaxis and abnormal hepatic function have been reported rarely.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE

In adults, somnolence, tachycardia and headache have been reported with overdoses greater than 10 mg. Extrapyramidal signs and palpitations have been reported in children with overdoses of greater than 10 mg. In the event of overdose, general symptomatic and supportive measures should be instituted promptly and maintained for as long as necessary. Treatment of overdose consist of emesis (ipecac syrup), except in patients with impaired consciousness, followed by the administration of activated charcoal to absorb any remaining drug. If vomiting is unsuccessful, or contraindicated, gastric lavage should be performed with normal saline. Saline cathartics draw water into bowel by osmosis and therefore, may be of value for rapid dilution of bowel contents. Loratadine is not eliminated by hemodialysis. It is not known if loratadine is eliminated by peritoneal dialysis.

SHELF- LIFE : The expiry date is indicated on the packaging.

STORAGE : Store below 30°C. Protect from moisture.

KEEP OUT OF REACH OF CHILDREN / JAUHI DARI KANAK-KANAK

DESCRIPTION

White to off white oval convex tablet, engraved with "XS" and break line on one side and plain on the reverse.

PACKING / PACK SIZES : Blister pack of 10's in packing of 10's, 100's and 1000's per box. Not all pack size(s) are available.

Revision Date : 01-Aug-2023



Manufacturer and Product Registration Holder (in Malaysia)

Xepa-Soul Pattinson (Malaysia) Sdn Bhd Company No. 8504-M 1-5 Cheng Industrial Estate, 75250 Melaka, Malaysia.